



HealthCare

CH
800
Lexington KY 40536-
0536
Notes Report

Sex: M
Adm: 3/18/2025, D/C: —

Legal

MRN: 110834266



Safdar, Rafeh, MBBS
Fellow
Oncology

Progress Notes  
Attested

Date of Service: 4/7/2025 2:17 PM

Attestation signed by Villano, John L, MD at 4/8/2025 9:01 AM

I saw and evaluated the patient with the resident/fellow. I discussed the case with the resident/fellow and agree with the findings and plan as documented.

Medical Oncology Progress Note

Admit Date: 3/18/2025
LOS: 20

Onc dx: Stage IV lung adenocarcinoma
Admit dx: Brain mets AMS
Disease status: Stage IV
Primary oncologist: F/U at Corbin after finishing radiation

---Assessment and Plan---

██████████ is a 66 y.o. male with a history of hypertension, dyslipidemia, CAD, type 2 diabetes, was transferred from outside hospital due to right hip pain and found to have brain metastases, patient found to have multiple metastatic brain lesions with the largest being in the right occipital and left frontal, In addition due possible lesions of the right frontal superior and inferior Gyri. Patient underwent surgical resection March 19, course complicated with a right-sided hemiplegia. CAT scan of the chest abdomen and pelvis revealed right upper lobe lung mass, post R lung nodules, bilateral adrenal nodules and 16 mm nodule in the right side of the liver admission to serosal chronic metastases.

Oncologic relevant problem list:

1. Stage IV poorly differentiated large cell adenocarcinoma, PDL1> 95%. Course complicated with a right-sided hemiplegia after surgical resection of the left thalamic lesion.
2. Indeterminate right liver nodule measuring 16 mm likely metastatic body due to cirrhotic liver morphology, recommended triphasic CAT scan or MRI.
3. Right sided hemiplegia. Might not have meaningful recovery.

Recommendations and plan:

- Plan for SAR on discharge.
- Agree with tapering dexamethasone to 2 mg BID, as per NSGY.
- Follow up with Rad Onc in Corbyn in outpatient setting, GKRS preferred for limited tumor board.
- **Medical oncology referral (in Corbyn per patient preference) after completes radiation oncology management to brain locally. Patient to receive ICI monotherapy given high PD-L1 expression.**

Medical oncology will sign off. Please call with questions or concerns.

Case discussed with Dr. Villano who was involved in the formulation of the plan above.

Rafeh Safdar, MBBS
PGY-4 Fellow, Hematology/Oncology
Markey Cancer Center

Pager: (859) 330-0731

--Subjective--**24 Hour Events:**

None.

Subjective:

Seen and examined at bedside. Sister present in room. Patient reports overall okay health, although communication more limited today. Denies any active health concerns.

--Objective--**Physical Exam:****Visit Vitals**

BP	123/52 (BP Location: Left arm, Patient Position: Lying)
Pulse	90
Temp	36.6 °C (97.8 °F) (Axillary)
Ht	1.702 m (5' 7.01")
Wt	84.3 kg (185 lb 13.6 oz)
SpO2	93%
BMI	29.10 kg/m ²

Lying in no acute distress. Chronically ill-appearing. Able to engage in interview without difficulty.

HEENT: No scleral icterus or conjunctival pallor. Mucous membranes moist.

Neck: Supple, no lymphadenopathy.

Cardiovascular: Capillary refill normal, pulses palpable.

Respiratory: Respiratory effort normal.

Abdomen: Soft, non-tender, non-distended.

Neurologic: No focal deficits.

Skin/MSK: RUE/RLE weakness, appears stable.

Labs, Imaging, and Ancillary Data:

All interim data independently reviewed in Epic by me, notable as in Assessment and Plan.

Cosigned by: Villano, John L, MD at 4/8/2025 9:01 AM

Electronically signed by Safdar, Rafeh, MBBS at 4/7/2025 2:25 PM

Electronically signed by Villano, John L, MD at 4/8/2025 9:01 AM

ED to Hosp-Admission (Current) on 3/18/2025 *Note shared with patient*

Care Timeline

03/20 1813 Admitted from ED 1813